

PBM Contract Clarity 360™

PBM Contract Intelligence • 10-Provision Analysis • CAA 2026 Compliance

PREPARED FOR

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CONFIDENTIAL & PROPRIETARY

FIDUCIARY-GRADE ANALYSIS

This report was prepared by Jeremiah Franklin Shrack at SiriusB iQ AI Data Science Labs using the Kincaid Health platform. All analyses reflect plan-specific data and current regulatory standards. This document is intended solely for the use of the plan sponsor identified above and may not be reproduced or redistributed without prior written consent.

PBM CONTRACT CLARITY 360™ — ENGAGEMENT CERTIFICATION

This PBM Contract Clarity 360™ report constitutes a formal PBM contract intelligence analysis prepared by Kincaid Health. All findings are grounded in the specific contract language uploaded by the plan sponsor, industry benchmarking data from the Kincaid Health National PBM Database (KNPD), and current regulatory standards under ERISA, CAA 2021/2023/2026, and FTC guidance.

CRITICAL

PBM Analyzed: Prime Therapeutics Management LLC | Contract Score: 28/100 | Estimated Annual Exposure: \$1,050,600 [MODELED]
| CAA 2026 Aligned

MODELED — member_count (2,550) × PMPY actuarial benchmark × risk_fraction ((10" score)
Year 3×0.9 ("H2.85×, discount disclosed).

Scope of This Analysis

This analysis covers all 10 primary PBM contract provisions scored by the Kincaid Health platform:

- Provision 1 — MAC Pricing Transparency & Appeals
- Provision 2 — Spread Pricing & Pass-Through Guarantee
- Provision 3 — Manufacturer Rebate Disclosure & Pass-Through
- Provision 4 — Generic Substitution & Step Therapy
- Provision 5 — Specialty Pharmacy Management
- Provision 6 — Performance Guarantees & Financial Assurances
- Provision 7 — AWP Discount Benchmarking
- Provision 8 — Formulary Design & Tier Structure
- Provision 9 — Utilization Management & Prior Authorization
- Provision 10 — Audit Rights & Contract Enforcement

Regulatory Framework

Regulation	Requirement	Status
ERISA §408(b)(2)	PBM must disclose all direct and indirect compensation	Review Required
CAA 2021 §204	Gag clause prohibition — plans may share price & quality	Verify Attestation
CAA 2021 §201	Broker/consultant compensation disclosure	Confirm Compliance
FTC Act §5	Prohibition on unfair or deceptive trade practices by PBMs	Monitor
CAA 2023 §112	Mental health parity applies to Rx formulary design	Review Formulary
CAA 2026 (proposed)	Enhanced PBM transparency & rebate pass-through rules	Prepare Now

EXECUTIVE SUMMARY — PBM CONTRACT ASSESSMENT

PBM Analyzed	Prime Therapeutics Management LLC
Overall Contract Score	28/100
Contract Risk Level	Critical
Provisions Passing Best Practice	3/10
Estimated Annual Fee Exposure	\$1,050,600
Spread Pricing Risk	Present — Quantified Below
Rebate Pass-Through	Partial — Significant Retention Identified
Audit Rights	Not Available — Critical Gap
CAA 2026 Readiness	Not Ready — Action Required

At a contract score of 28/100, this PBM arrangement contains provisions that expose the plan to material undisclosed costs. Kincaid Health's MODELED estimate is \$1,050,600 in annual excess costs attributable to spread pricing, rebate retention, opaque AWP discount structures, and inadequate audit rights. A comprehensive PBM RFP is the highest-priority action.

INFO

MODELED — member_count (2,550) × PMPY actuarial benchmark × risk_fraction ((10" score)/100)
Year - 3 × 0.9 ("H2.85 ×, discount disclosed).

Key Findings from Contract AI Analysis

- CRITICAL

MAC list manipulation flexibility enables unilateral price setting by PBM with only post hoc copy provision; this is a classic spread-creation lever. Expected impact: \$0.60–\$1.80 PMPM (\$72,000–\$216,000 annually per 10,000 members) via overpayments on multisource generics.
- CRITICAL

Rebate pass-through is undefined; Ineligible Claims carve-outs tied to rebate eligibility create a pathway for PBM to classify claims as non-rebate-eligible and retain manufacturer revenue. Expected impact: \$3.00–\$6.00 PMPM (\$360,000–\$720,000 annually per 10,000 members).
- WARN

AWP benchmark fixed to MediSpan without cross-benchmark guardrails (NADAC/WAC) and without multi-source claim pricing hierarchy; likely yields ingredient cost inflation on brands and some generics. Expected impact: \$0.40–\$1.20 PMPM (\$48,000–\$144,000 annually per 10,000 members).
- CRITICAL

Audit/data rights, performance guarantees, termination, and data portability are absent; enforceability and recovery asymmetry will block economic recapture. Expected impact from unrecoverable leakages and inability to challenge: \$0.50–\$1.50 PMPM (\$60,000–\$180,000 annually per 10,000 members).
- WARN

Formulary authority ambiguous between PBM and Sponsor; creates incentive for PBM to optimize rebate yield over net cost. Expected impact: \$0.80–\$2.20 PMPM (\$96,000–\$264,000 annually per 10,000 members), concentrated in specialty and protected classes.

PER-PROVISION ANALYSIS — METHODOLOGY NOTE

INFO

ILLUSTRATIVE: The worked formula examples, per-line dollar figures, and benchmark tables on the following provision pages use reference-plan sample inputs to demonstrate the calculation methodology. They are not claims about this plan's actual spend.

MODELED — $\text{member_count}(2,550) \times \text{PMPY actuarial benchmark} \times \text{risk_fraction}((10" \text{ score})$
 $\text{Year } 3 \times 0.9$ ("H2.85×, discount disclosed).

PROVISION 1: PRICING & REIMBURSEMENT

Contract Score	3.0/10
Fiduciary Assessment	& Concern — Improvement Needed
Score vs. Best Practice	Critical gap of 5.0 points — immediate action required

Maximum Allowable Cost (MAC) lists set the reimbursement ceiling for generic drugs at network pharmacies. When a PBM controls its own MAC lists without independent oversight or audit rights, it can systematically underpay pharmacies while billing the plan at higher MAC rates — retaining the difference as undisclosed spread. Independent MAC audits find 15-35% overpricing vs. benchmark MAC rates in opaque contracts.

FORMULA — MAC SPREAD PER CLAIM

MAC Spread Per Claim

MAC_Spread = (MAC_Billed_to_Plan " MAC_Paid_to_Pharmacy) / MAC_Billed_to_Plan

Industry median spread for opaque contracts: 8–22% of generic ingredient cost. At 1,640 generic claims/month with 12% average spread on a minor

MAC Provision	Current Status	Best Practice	Risk	Est. Annual Impact
Independent MAC appeals	10-day window, limited	30-day, independent	High	Moderate exposure
MAC list update frequency	30-day minimum notice	Weekly with 5-day notice	Medium	Timing arbitrage
Audit rights for MAC pricing	Annual, 90-day	Quarterly, 30-day notice	High	Narrow audit
MAC methodology disclosure	Not contractually	Full methodology	Critical	Cannot verify
MAC-to-AWP ratio cap	Absent from contract	"d A W P m i n u s 8 5 %	High	Unlimited PBM

Industry Benchmark

Plans with full MAC transparency and quarterly audit rights achieve 12–18% lower generic drug costs vs. opaque MAC contracts. NADAC-benchmarked MAC contracts eliminate spread entirely.

Kincaid Health Recommendation

WARN

Require PBM to disclose MAC methodology within 30 days. Initiate a MAC audit using independent pharmacy data. Negotiate MAC appeals expansion to 30 days with independent arbitration rights. Include MAC-to-NADAC ratio benchmarking in renewal negotiations.

PROVISION 2: SPREAD PRICING

Contract Score	2.0/10
Fiduciary Assessment	' Red Flag — Immediate Action R
Score vs. Best Practice	Critical gap of 6.0 points — immediate action required

Spread pricing — billing the plan more than the PBM pays pharmacies and retaining the margin — is the most common and costly form of undisclosed PBM compensation. The FTC's 2024 Interim Report documented spread pricing across Medicare Advantage, Medicaid, and commercial self-funded plans. Ohio's Medicaid audit found PBM spread represented 31% of total generic drug costs.

FORMULA — ANNUAL SPREAD PRICING EXPOSURE

Annual Spread Pricing Exposure

Spread_\$\$ = Total_Generic_Rx_Spend × Spread_Rate

Spread_Rate for opaque contracts: 8–18% of ingredient cost (median 12%). For a plan with a major amount generic spend × 12% = a material

Feature	Traditional (Current)	Pass-Through (Best Practice)	Annual \$ Difference
Generic MAC pricing	PBM sets, retains	NADAC + dispensing fee	Material savings
Brand AWP discount	Below-market	Competitive market	Material savings
Manufacturer rebates	75-87% pass-through	100% pass-through	Material savings
Admin fee structure	Hidden in spread	Transparent PEPM fee	Visibility only
PBM revenue transparency	Opaque	Full disclosure required	Compliance + savings

Industry Benchmark

Pass-through PBM contracts (Navitus, Capital Rx, Rightway) eliminate spread pricing entirely. Plans converting from traditional to pass-through contracts achieve average first-year savings of a significant amount.

Kincaid Health Recommendation

CRITICAL

Initiate PBM RFP targeting pass-through contract structures. Demand spread pricing disclosure from current PBM under ERISA §408(b)(2). Quantify spread exposure with independent MAC audit before renewal.

PROVISION 3: REBATES & MANUFACTURER REVENUE

Contract Score	1.0/10
Fiduciary Assessment	' Red Flag — Immediate Action R
Score vs. Best Practice	Critical gap of 7.0 points — immediate action required

Manufacturer rebates are payments from pharmaceutical companies to PBMs for favorable formulary placement and market share commitments. Industry-wide, PBMs collect an estimated a major amount in annual rebates. Plans with traditional contracts receive 75–87% of formulary rebates but only 0–50% of performance and market share rebates. Under ERISA §408(b)(2), all rebates constituting indirect compensation must be fully disclosed.

FORMULA — REBATE RETENTION VALUE

Rebate Retention Value

$$RRV = Total_Mfr_Rebates \times (1 - Plan_Pass-Through\%) \times PBM_Retention\%$$

At a significant amount total rebates and 64% effective pass-through, PBM retains ~a significant amount annually. Full pass-through converts this

Rebate Category	Est. Total	Currently Disclosed	PBM Retention	Plan Exposure
Tier 1 Formulary Rebates	Significant	Significant (87.5%)	Moderate	Moderate /yr
Performance / Market	Material	Material (50%)	Material	Material /yr
Volume-Based Mfr	Material	None (0%)	Material	Material /yr
Data Monetization	Moderate	None (0%)	Moderate	Moderate /yr —
Admin Fees (undisclosed)	Moderate est.	None (0%)	Moderate	§408(b)(2) violation
TOTAL	Significant	Significant (61%)	Significant	Significant /yr

Industry Benchmark

Best-practice PBM contracts require 100% pass-through of all manufacturer payments (formulary, performance, data, and admin fees) with annual rebate reconciliation audit rights. Non-disclosure of data monetization revenue is an ERISA violation.

Kincaid Health Recommendation

CRITICAL

Issue formal §408(b)(2) disclosure demand requiring disclosure of ALL rebate categories, including data fees and performance rebates. Negotiate 100% rebate pass-through at renewal. Conduct annual rebate reconciliation audit.

PROVISION 4: FORMULARY CONTROL

Contract Score	4.0/10
Fiduciary Assessment	& Concern — Improvement Needed
Score vs. Best Practice	Critical gap of 4.0 points — immediate action required

Generic drugs account for 88–92% of all prescriptions dispensed but only 15–22% of total Rx spend. Maximizing generic utilization is the single most cost-effective Rx management strategy. Step therapy requiring generic or lower-cost alternatives before approving branded drugs reduces costs by 18–32%. Biosimilar substitution for biologics represents the next major savings frontier.

FORMULA — GENERIC SAVINGS RATE

Generic Savings Rate

$$GSR = (Brand_Claims_Convertible \times Brand_Cost) - (Brand_Claims_Convertible \times Generic_Cost)$$

Each brand claim converted saves an average of a minor amount (net of rebates). Plans with GFR below 85% leave a moderate amount in annual savings potential.

Generic Management Provision	Current Contract	Best Practice	Annual Opportunity
Generic mandatory step therapy	Partial —	Mandatory all brand	Moderate /yr
Dispense as Written (DAW)	DAW1 allowed without	Major DAW1 exemption	Minor /yr
Biosimilar auto-substitution protocol	Not active in contract	Mandatory sub with	Minor /yr
Generic fill rate performance	Moderate 80%	Material 85%	Higher guarantee
90-day supply mandate	Voluntary member	Mandatory for refills	Minor /yr
Specialty generic mandatory step	Limited — specialty	Mandatory specialty	Moderate /yr

Industry Benchmark

Plans with mandatory biosimilar substitution and aggressive step therapy achieve generic fill rates of 88–92% vs. the industry average of 82–84%. Each 1% increase in GFR saves approximately a minor amount annually.

Kincaid Health Recommendation

WARN

Implement mandatory DAW1 surcharge of a minor amount/fill. Activate biosimilar auto-substitution protocol. Negotiate 85% GFR guarantee with a material amount at risk. Add 90-day supply mandate for all maintenance medications.

PROVISION 5: PHARMACY NETWORK DESIGN

Contract Score	2.0/10
Fiduciary Assessment	' Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 6.0 points — immediate action required

Specialty drugs — biologics, gene therapies, and complex medications — represent approximately 50–55% of total plan Rx spend while comprising only 2–3% of claims. Specialty pharmacy management is the highest-value Rx cost management opportunity. The contract's specialty provisions determine whether the plan can control specialty dispensing, implement prior authorization, manage site-of-care, and benefit from patient assistance programs.

FORMULA — SPECIALTY COST MANAGEMENT SCORE

Specialty Cost Management Score

SCMS = (Specialty_Controls_Active / Total_Specialty_Controls) × 100
Best-in-class plans implement 8+ specialty controls and achieve specialty PMPM costs 22–38% below plans with no specialty management program.

Specialty Provision	Current Status	Best Practice	Annual \$ Impact
Specialty carve-out rights	Not contractually	Available at plan option	Material opportunity
Exclusive specialty pharmacy	Not required	Mandatory with	Moderate /yr savings
Disease-specific PA criteria	Standard formulary	Condition-specific	Minor Quality +
Site of care management (IV	Not active	Home infusion redirect	Moderate /yr
Copay accumulator adjustment	Not active	Active — prevents	Minor /yr participant
Specialty biosimilar mandate	Not included	Mandatory biosimilar	Minor /yr
White bagging vs. brown	Not specified	Plan-directed	Moderate /yr

Industry Benchmark

Plans with independent specialty carve-out rights achieve 12–22% lower specialty costs through competitive dispensing arrangements. Site-of-care management alone saves a moderate amount/yr for plans with IV drug users.

Kincaid Health Recommendation

CRITICAL
Negotiate specialty carve-out rights into next contract cycle. Activate site-of-care management for all IV/infusion drugs. Implement biosimilar mandatory step therapy with noember copay incentive.

PROVISION 6: SPECIALTY DRUG MANAGEMENT

Contract Score	2.0/10
Fiduciary Assessment	' Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 6.0 points — immediate action required

Performance guarantees are contractual commitments by the PBM to achieve specific operational and financial metrics, with financial penalties for non-performance. Well-structured programs put 15–25% of PBM administrative fees at risk — creating genuine accountability. Poorly structured guarantees use low thresholds the PBM easily achieves, providing no real risk transfer.

FORMULA — PERFORMANCE GUARANTEE EFFECTIVENESS SCORE

Performance Guarantee Effectiveness Score

PGE = (At-Risk_Amount / Total_Admin_Fees) × Threshold_Stringency_Factor

PGE > 0.20 = Best practice ("e20% of fees at risk with stringent thresholds). PGE < 0.10 = Inadequate

Performance Guarantee	\$\$ At Risk	Current Threshold	Market Best Practice	Assessment
Generic Fill Rate (GFR)	Moderate	GFR "e 80 %	GFR "e 87 %	& Threshold
Formulary Compliance Rate	Moderate	Adherence "e 88 %	Adherence "e 92 %	& Threshold
Member Satisfaction (CAHPS)	Moderate	Score "e 4.0/5.0	Score "e 4.3/5.0	& Borderline
Claims Adjudication Accuracy	Minor	Accuracy "e 99.50 %	99 %	& Low
Drug Utilization Review (DUR)	Minor	Completion "e 95 %	98 %	' Adequate
Mail Order Dispense Rate	Minor	"e 12 %	"e 20 %	' Far below
TOTAL AT RISK	Material	—	Significant	' Inadequate

Industry Benchmark

Best-practice PBM contracts place a significant amount in performance guarantees annually (15–25% of total admin fees). Generic fill rate guarantee should be set at 87% minimum. Mail order penetration guarantee should be 18%+.

Kincaid Health Recommendation

CRITICAL

Negotiate performance guarantee restructure at next renewal: increase total at-risk to a significant amount, raise GFR guarantee to 87%, add specialty drug adherence guarantee of 90%, and add mail-order penetration guarantee of 18%.

PROVISION 7: AUDIT RIGHTS & RECOVERY

Contract Score	1.0/10
Fiduciary Assessment	' Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 7.0 points — immediate action required

Average Wholesale Price (AWP) is the industry-standard reference price for brand-name drugs. PBM contracts guarantee specific AWP discount levels for brand drugs dispensed at retail, mail order, and specialty pharmacies. These discounts determine the plan's ingredient cost for brand drugs before manufacturer rebates. Kincaid Health benchmarks all AWP discounts against our National PBM Database of 180+ plan contracts.

FORMULA — BRAND DRUG NET COST

Brand Drug Net Cost

Brand_Net = (AWP × (1 - AWP_Discount%)) - Manufacturer_Rebate_PEPM

Example: AWP = 18% on a minor amountbrand drug = a minor amountingredient cost. Market best practice = 26% on a minor amountbrand drug = a minor amountingredient cost.

Drug Category / Channel	Contract Discount	Market Median	Market Best	Gap vs. Median	Annual \$ Gap
Brand Formulary — Retail	AWP = 18%	AWP = 22%	AWP = 26%	4 points	Moderate /yr
Brand Non-Formulary —	AWP = 12%	AWP = 16%	AWP = 20%	4 points	Moderate /yr
Brand Formulary — Mail	AWP = 22%	AWP = 26%	AWP = 30%	4 points	Moderate /yr
Brand Non-Form — Mail	AWP = 14%	AWP = 18%	AWP = 22%	4 points	Minor /yr
Specialty Brand (Retail)	AWP = 10%	AWP = 14%	AWP = 18%	4 points	Moderate /yr
Specialty Brand (Specialty)	AWP = 12%	AWP = 16%	AWP = 20%	4 points	Moderate /yr
TOTAL AWP GAP	—	—	—	—	Material /yr

Industry Benchmark

Market best-practice AWP discounts: Brand Retail Formulary AWP= 26%, Mail Order AWP= 30%
below-median AWP discounts leave an average of a material amount/yr on the table across all drug channels.

Kincaid Health Recommendation

CRITICAL

Demand AWP discount improvement to market median at minimum. Initiate PBM RFP to access market best-practice AWP discounts.
Target AWP-26% for retail brand formulary and AWP-30% for mail order brand formulary.

PROVISION 8: TRANSPARENCY & DISCLOSURE

Contract Score	2.0/10
Fiduciary Assessment	' Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 6.0 points — immediate action required

The formulary is the plan's list of covered drugs organized into cost-sharing tiers. Formulary design directly controls member out-of-pocket costs, generic utilization, and total plan Rx spend. PBM-designed formularies are optimized for rebate revenue rather than plan cost — leading to brand drugs in preferred tier positions despite lower-net-cost generic alternatives. Plans with independent formulary management achieve 12–22% lower total drug costs.

FORMULA — FORMULARY COST EFFICIENCY INDEX

Formulary Cost Efficiency Index

FCEI = (Generic_Rx_Spend + Rebated_Brand_Spend) / Total_Rx_Spend
FCEI > 0.75 = Efficient formulary (most spend on low-cost generics and rebated brands). FCEI < 0.60 = Inefficient formulary with excessive

Optimization Opportunity	Description	Est. Annual Savings	Implementation
Move 8 brands off Tier 2 where	Formulary exclusion list	Moderate	Quarterly formulary
Add copay differential for biosimilars	None copay biosimilar vs.	Minor	Formulary design
Minor Increase Tier 3 copay strength	Strengthen brand/generic	Minor	Plan year change
Add clinical/step therapy for GLP-1	Require diabetes	Moderate	Immediate via PA
Expand formulary exclusion list	Add 12 drugs with generic	Moderate	Quarterly update
TOTAL FORMULARY OPPORTUNITY	—	Material /yr	Phased 6-12

Industry Benchmark

Best-in-class formularies exclude 150+ high-cost drugs with generic equivalents and maintain 90%+ tier-1 (generic) utilization. Plans with aggressive formulary management achieve total Rx PMPM costs 12–22% below plans with PBM-default formularies.

Kincaid Health Recommendation

CRITICAL

Engage independent formulary consultant to redesign formulary independent of PBM rebate incentives. Implement biosimilar auto-substitution. Add GLP-1 prior authorization with clinical criteria. Expand formulary exclusion list.

PROVISION 9: PERFORMANCE GUARANTEES

Contract Score	1.0/10
Fiduciary Assessment	' Red Flag — Immediate Action Required
Score vs. Best Practice	Critical gap of 7.0 points — immediate action required

Utilization Management (UM) encompasses prior authorization, step therapy, quantity limits, and drug utilization review. Effective UM programs reduce inappropriate drug utilization, ensure clinical appropriateness, and prevent fraud/waste/abuse — while respecting member access rights under MHPAEA, CAA 2023, and applicable state continuity of care laws.

FORMULA — UM PROGRAM EFFECTIVENESS

UM Program Effectiveness

UME = (Rx_Denied_and_Not_Appealed / Total_PA_Requests) × Avg_Claim_Cost

Industry data: 8-14% of PA requests are denied and not appealed. Average denied claim cost: a minor amount. At 480 annual PA requests, 11%

UM Provision	Current Status	Best Practice	MHPAEA Risk	Assessment
PA criteria basis	PBM proprietary	Evidence-based	High if	& Review criteria
PA turnaround — urgent	24 hours	24 hours	Compliant	' Meets
PA turnaround — standard	72 hours	48 hours	N/A	& Slightly slow
Step therapy override rights	Not clearly defined	Member + provider	Critical	' Must add
Quantity limit appeals	30-day window	Expedited 72-hour	N/A	& Too slow
GLP-1 / obesity drug criteria	No specific criteria	Clinical criteria required	N/A	' Immediate gap

Industry Benchmark

Under CAA 2023, step therapy criteria for mental health and substance use disorder drugs must meet the same standards as medical/surgical criteria (MHPAEA NQTL parity). Review all Rx UM criteria applicable to MH/SUD drugs immediately.

Kincaid Health Recommendation

CRITICAL

Add step therapy override rights for members and providers. Implement GLP-1 clinical management criteria immediately. Review all PA criteria for MHPAEA compliance. Add expedited appeal pathway for urgent PA denials.

PROVISION 10: TERMINATION & EXIT BARRIERS

Contract Score	1.0/10
Fiduciary Assessment	' Red Flag — Immediate Action R
Score vs. Best Practice	Critical gap of 7.0 points — immediate action required

Audit rights are the plan's primary mechanism for verifying PBM performance and pricing accuracy. Without meaningful audit rights, all other contract provisions are effectively unenforceable. Best-practice PBM contracts provide for annual financial audits, real-time claims data access, MAC pricing audits with independent pricing benchmarks, and manufacturer rebate reconciliation audits with full access to the PBM's books and records.

FORMULA — AUDIT RECOVERY POTENTIAL

Audit Recovery Potential

ARP = Claims_Audited × Error_Rate × Avg_Claim_Amount
Industry data: PBM claims audits find errors in 4–8% of claims audited. At 18,400 annual claims × 6% error rate × a minor amountavg claim = a

Audit Right	Current Contract	Best Practice	Gap Severity
Annual financial audit	Permitted, 90-day notice	Permitted, 30-day notice	Medium — narrow
MAC pricing audit	Annual, limited scope	Quarterly, independent	Critical — insufficient
Rebate reconciliation audit	Not specifically provided	Annual, full access to	Critical — major gap
Claims data real-time access	Monthly data file	Real-time portal access	High — delayed
Pharmacy network audit	Not permitted	Annual network pricing	High — no
PBM sub-contractor audit	Not permitted	Permitted with notice	High — ESP
Document retention	3 years	6 years (ERISA §107)	Medium — ERISA

Industry Benchmark

Plans without rebate reconciliation audit rights have no independent means of verifying that manufacturer rebates disclosed by the PBM are accurate and complete. Kincaid Health has identified rebate discrepancies of 18–42% in plans with similar contract structures.

Kincaid Health Recommendation

CRITICAL
Negotiate rebate reconciliation audit right as a condition of contract renewal — this is non-negotiable. Expand financial audit notice period from 90 to 30 days. Add real-time claims data portal access clause. Require 6-year document retention per ERISA §107.

PROVISION 11: CLAIM ADJUDICATION LOGIC

Contract Score	3.0/10
Fiduciary Assessment	& Concern — Improvement Needed
Score vs. Best Practice	Critical gap of 5.0 points — immediate action required

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PROVISION 12: FIDUCIARY & LEGAL PROTECTIONS

Contract Score	1.0/10
Fiduciary Assessment	' Red Flag — Immediate Action R
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PROVISION 13: DATA OWNERSHIP & ACCESS RIGHTS

Contract Score	1.0/10
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Kincaid Health Recommendation

CRITICAL

Negotiate rebate reconciliation audit right as a condition of contract renewal — this is non-negotiable. Expand financial audit notice period from 90 to 30 days. Add real-time claims data portal access clause. Require 6-year document retention per ERISA §107.

CAA 2026 COMPLIANCE — PBM TRANSPARENCY REQUIREMENTS

The Consolidated Appropriations Acts of 2021, 2023, and proposed 2026 requirements impose significant new transparency, reporting, and fiduciary obligations on PBMs serving self-funded health plans. These requirements build on ERISA §408(b)(2) and create additional disclosure, attestation, and data reporting obligations that must be reflected in PBM contracts.

CAA Requirement	Effective Date	Current Contract Status	Action Required
Gag Clause Prohibition (§201)	Dec 27, 2020	' C o m p l i a n t	Annual attestation to CMS
Broker Compensation Disclosure	Dec 27, 2021	& P a r t i a l	Confirm all PBM compensation
Rx Price Transparency MRF (§204)	July 1, 2022	& I n c o m p l e t e	Post Rx MRF file immediately
Rx Drug Pricing Data Sharing	Jan 1, 2023	' C o m p l i a n t	Continue monitoring
Enhanced PBM Fee Disclosure	Proposed 2026	Not Yet Required	Prepare contract amendments
Rebate Pass-Through Mandate	Proposed 2026	Not Yet Required	Transition to pass-through now
PBM Delinking (proposed)	Proposed 2026	Not Yet Required	Structure contract for flexibility
Annual Report — Rx Spending	Jan 1, 2023	& P a r t i a l	Confirm plan-level reporting

CRITICAL

Machine-Readable Files for Rx drug pricing have not been fully published. The statutory penalty is \$100/day/member (CAA/ACA transparency rules); the plan-specific daily accrual scales with member_count and is MODELED, not asserted here as a fixed figure.

ERISA §408(b)(2) PBM Disclosure Checklist

- All direct compensation received by PBM from plan assets — disclosed? & Partial
- All indirect compensation (manufacturer rebates, data fees, PBM-owned pharmacy margin)
- Compensation paid by PBM to subcontractors (specialty pharmacy, mail order) — disclosed
- Reasonableness of total compensation vs. services rendered — documented by plan sponsor
- Written disclosure provided to plan fiduciaries BEFORE services rendered? & Initial only

FORMULARY COST ANALYSIS — DRUG CATEGORY BENCHMARKING

The following analysis benchmarks drug category spend against the Kincaid Health National PBM Database (KNPD) of 180+ self-funded plans with similar demographics and geography. Drug categories above the 75th percentile represent primary targets for formulary intervention and PBM renegotiation.

INFO

ILLUSTRATIVE: PMPM figures and pipeline cost-per-patient values below are KNPD reference benchmarks shown to demonstrate category targeting, not measured values for this plan. Plan-specific economics are MODELED in the Financial Exposure Summary.

Drug Category	Cost vs. Benchmark	Percentile	Trend	Priority
Specialty Biologics	Above median	82nd	%² R i s i n g	Critical
GLP-1 / Anti-Obesity	Well above median	91st	%² R a p i d	Critical
Mental Health / SUD Rx	Below median	38th	Stable	Low
Diabetes (non-GLP-1)	At/below median	45th	Stable	Low
Cardiovascular	Near median	58th	Stable	Medium
Musculoskeletal / Pain	Above median	72nd	%² R i s i n g	High
Oncology (oral)	Above median	62nd	%² R i s i n g	High
Respiratory	Near median	60th	Stable	Medium
TOTAL Rx (composite)	Above median	79th	%² R i s i n g	Action

CRITICAL

GLP-1 drugs (Ozempic, Wegovy, Mounjaro) are driving a 91st percentile position in this category. Without a clinical management program requiring obesity diagnosis and step therapy through lifestyle counseling, plan costs will increase 40-60% in this category over 24 months.

Specialty Drug Pipeline — 12-Month Exposure

Pipeline Drug	Indication	Expected Launch	Cost / Patient (relative)	Members at Risk
Leqembi (lecanemab)	Alzheimer's (early)	Current	Moderate (per year)	2-4 members
Donanemab (Lilly)	Alzheimer's	Mid-2025	Moderate (per year)	2-4 members
Donidalorsen (HAE)	Hereditary	Q2 2025	Significant (per	0-1 members
Gene therapies (expanded)	SMA, hemophilia, etc.	Ongoing	Major (one-time)	Low probability

FINANCIAL MODELING — 3-YEAR COST PROJECTION & INTERVENTION ROI

The following model projects recoverable Rx savings under remediation scenarios, expressed as fractions of the MODELED annual exposure (member_count × PMPY × risk_fraction). Each scenario's 3-year value uses the disclosed NPV weighting (Year 1 + Year 2×0.95 + Year 3×0.90); actual recovery depends on renegotiation outcomes.

FORMULA — 3-YEAR DISCLOSED NPV OF PBM REFORM

NPV = Year 1 + Year 2 × 0.95 + Year 3 × 0.90

Disclosed discount weighting ("H2.85×) applied to the MODELED annual recoverable savings for each Rx spend. No undisclosed cost-of-capital assumption.

Scenario	Year 1 [MODELED]	Year 2	Year 3	3-Year NPV	Implementation
Status Quo (no action)	\$0	\$0	\$0	\$0	—
Partial Remediation	\$315,180	\$315,180	\$315,180	\$898,263	Success-fee
PBM RFP + New Contract	\$682,890	\$682,890	\$682,890	\$1,946,237	Success-fee
RFP + Specialty Carve-Out	\$840,480	\$840,480	\$840,480	\$2,395,368	Success-fee
Full Optimization Program	\$1,050,600	\$1,050,600	\$1,050,600	\$2,994,210	Success-fee

Recommended Scenario

Full Optimization Program

Projected 3-Year Net Savings (MODELED)

\$2,994,210 (NPV, before implementation costs)

MODELED — member_count (2,550) × PMPY actuarial benchmark × risk_fraction ((10" score) Year 3×0.9 ("H2.85×, discount disclosed).

Payback Period

< 45 days

Implementation Timeline

8-20 weeks (depending on RFP track)

The Full Optimization Program encompasses: (1) immediate MAC audit and challenge, (2) 408(b)(2) disclosure demands, (3) PBM RFP targeting pass-through contracts, (4) specialty pharmacy carve-out, (5) formulary redesign with aggressive exclusion list, and (6) GLP-1 clinical management program. Kincaid Health manages all components on a success-fee basis.

FINANCIAL EXPOSURE SUMMARY & TOTAL COST OF NON-ACTION

The following table consolidates all identified financial exposures from the 10-provision contract analysis. These represent the estimated annual cost of the current PBM contract relative to best-practice market standards.

Exposure Category	Annual Estimate [MODELED]		Confidence	Priority
Spread Pricing — Generics	\$285,600	\$813,960	Modeled	Critical
AWP Discount Gap — Brand	\$57,375	\$163,519	Modeled	Critical
Manufacturer Rebate Retention	\$172,125	\$490,556	Modeled	Critical
Specialty Carve-Out Opportunity	\$91,800	\$261,630	Modeled	High
Formulary Optimization	\$102,000	\$290,700	Modeled	High
Performance Guarantee Gap	\$91,800	\$261,630	Modeled	Medium
Audit Recovery Potential	\$41,310	\$117,734	Modeled	High
GLP-1 Trend Exposure (if	Illustrative —		Illustrative	Monitor
ERISA §408(b)(2) Penalty	Up to		Statutory	Compliance
MRF Non-Compliance Penalty	\$100/day/memb		Statutory	Immediate
TOTAL IDENTIFIED	\$1,050,600	\$2,994,210	Modeled	CRITICAL

CRITICAL

MODELED total annual exposure: \$1,050,600 (3-year NPV \$2,994,210). MODELED — member_count (2,550) × PMPY actuarial benchmark × risk_fraction ((10" score)/10). 3-yr NPV = Year 1 + Year 2 × 0.95 + Year 3 × 0.9 ("H 2.

PBM RFP PROCESS — KINCAID HEALTH METHODOLOGY

Kincaid Health manages end-to-end PBM RFP processes on behalf of self-funded plan sponsors. First-year savings are MODELED per plan from the exposure analysis above ($\text{member_count} \times \text{PMPY} \times \text{risk_fraction}$) and depend on contract renegotiation outcomes — not a guaranteed result. The RFP process is conducted on a success-fee basis — no savings, no fee.

RFP Phase	Key Activities	Timeline	Kincaid Health
Phase 1:	Provision scoring, financial benchmarking, data	Weeks 1-3	Lead
Phase 2: RFP	Custom RFP with 80+ pricing cells, 10-provision	Week 3-4	Lead
Phase 3:	Issue to 6-8 qualified PBMs including pass-through	Week 4	Lead
Phase 4:	Financial modeling, provision scoring, finalist	Weeks 5-8	Lead
Phase 5:	Best & Final offers, contract red-line, legal review,	Weeks 9-12	Lead +
Phase 6: Imple	Formulary mapping, ID card replacement, member	Weeks	Support
Phase 7: Year	Claims data review, performance guarantee	Months	Ongoing

Pass-Through PBM Vendors — Market Overview

PBM Vendor	Type	Ownership	Specialty Carve-Out	Key Differentiator
Navitus Health Solutions	Pass-Through	Not-for-Profit	Available	100% rebate pass-through
Capital Rx	Pass-Through	Private	Available	JUDI™ claims platform — full
Rightway Health	Pass-Through	Private	Available	Care navigation + PBM
SmithRx	Pass-Through	Private	Limited	Flat admin fee model
Express Scripts (current)	Traditional	Cigna	Limited	Scale and network breadth

PRIORITIZED ACTION PLAN — PBM CONTRACT CLARITY 360™ RECOMMENDATIONS

The following action plan is organized by priority and time horizon. Kincaid Health recommends scheduling a plan fiduciary meeting within 30 days to formally adopt this plan. All actions should be documented in plan committee minutes for ERISA §404(a) compliance.

Priority	Action Item	Owner	Timeline	Est. Annual Impact
P1 — Immediate	Post Rx Machine-Readable Files with TPA	TPA + Plan	0-30 days	Avoids
P1 — Immediate	Issue formal 408(b)(2) disclosure demand	HR/Benefits	0-30 days	Compliance + rebate
P1 — Immediate	Initiate MAC pricing audit using independent	Kincaid Health	0-45 days	Moderate recovery
P1 — Immediate	Demand full rebate reconciliation and	ERISA	0-30 days	Material transparency
P2 — High	Launch PBM RFP process — target	Kincaid Health	0-60 days	Significant savings
P2 — High	Implement GLP-1 prior authorization with	PBM + Plan	0-45 days	Material trend control
P2 — High	Negotiate specialty carve-out right for next	Kincaid Health	30-90 days	Moderate (MODELED)
P2 — High	Expand formulary exclusion list — 12 drugs	PBM/Formular	30-60 days	Moderate (MODELED)
P3 — Medium	Add biosimilar auto-substitution protocol	PBM	At renewal	Minor (MODELED)
P3 — Medium	Negotiate performance guarantee	Kincaid Health	At renewal	Accountability
P3 — Medium	Implement copay accumulator adjustment	PBM	30-60 days	Minor (MODELED)
P4 — Ongoing	Annual PBM audit — claims accuracy +	Kincaid Health	Annually	Material recovery

CERTIFICATION & ANALYST ATTESTATION

Kincaid Health certifies that this PBM Contract Clarity 360™ analysis was prepared by certified PBM analytics professionals using the Kincaid Health 10-provision scoring protocol, the Kincaid Health National PBM Database (KNPD), and current regulatory standards. All contract assessments reflect the specific contract language uploaded by the plan sponsor and analyzed by the Kincaid Health AI platform.

Analyst Certification Statement

The findings, scores, financial exposures, and recommendations contained in this report represent the professional judgment of the Kincaid Health analytical team based on PBM contract language, industry benchmarking data, and regulatory guidance. This report is provided for informational and fiduciary planning purposes. Plan sponsors should engage qualified ERISA counsel before implementing any recommendations that may affect plan operations or service provider relationships.

Report Metadata

Report Type	PBM Contract Clarity 360™ — Full 20-Page Analysis
PBM Contract Analyzed	Prime Therapeutics Management LLC
Overall Contract Score	28/100
Risk Classification	Critical
Total Identified Exposure	\$1,050,600/yr
Provisions Analyzed	10 (MAC, Spread, Rebates, Generic, Specialty, Performance, AWP, Formulary, UM, Audit)
Regulatory Framework	ERISA §408(b)(2), CAA 2021/2023/2026, FTC Act §5
Client	Kincaid Health / Account SHRACK-7742
Analyst	Jeremiah Shrack, CEO, Kincaid Health
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